SAMPLE ANSWER · DISTAL RADIUS FRACTURE · CASE 1

Sample 7-minute ISBAR: Pat

Patricia Morgan · 68-year-old woman · Outpatient hand therapy clinic, public hospital · about 1044 spoken words (≈ 7.5 min at 140 words per minute)

I Introduction

81 words

"Hi, I'm the occupational therapy student who has been seeing Patricia Morgan, known as Pat, in hand therapy clinic today. Pat is a 68-year-old woman who is eight days after plate fixation of a right distal radius fracture.

"I need to discuss her with you urgently, because she has new and worsening numbness in her thumb, index and middle fingers, and I don't think we should go ahead with routine splinting and exercises until the orthopaedic team has reviewed her today."

S Situation

143 words

"Pat came in for backslab removal, a custom thermoplastic splint and to start the surgeon's routine early-motion programme.

"However, Pat tells me the tips of her right thumb, index and middle fingers became tingly yesterday afternoon and now feel 'like cotton wool', while her ring and little fingers feel normal. She describes a burning pain, different from her bone pain, which kept her awake and wasn't relieved by two of her strong painkillers at 4 am. Her fingers are puffy and shiny, the crepe bandage was tight enough to leave deep ridges, and she struggled to bring her thumb across to her ring finger.

"Her husband's parking is about to expire, my next patient has arrived, and Pat is asking me to just fit the splint so she can go. I've unwrapped the bandage, but I haven't fitted anything or started any exercises."

B Background

174 words

"Pat is right-handed and fell on her outstretched hand at her lawn bowls club. The fracture was comminuted and extended into the joint, and it was fixed with a volar locking plate eight days ago. On discharge, her wound was clean and she was neurovascularly intact, and she went home on day one with paracetamol and a short course of oxycodone.

"Since then, she has been sleeping with the arm hanging over the side of the bed and stopped using her sling at the weekend. Her husband, Graham, says she's 'not herself'.

"Before the fall, she was fully independent, driving, playing bowls twice a week and minding her two-year-old granddaughter three days a week.

"There are some important gaps. The surgeon's letter was written on the day of discharge, there are no neurovascular observations documented since she went home, and we don't know whether she had any numbness before surgery. The letter doesn't state her movement or loading restrictions beyond 'as per protocol', and I'm not sure she was told which symptoms to report."

A Assessment

300 words

“My main concern is that Pat’s symptoms follow the distribution of the median nerve, and they are new and progressing. Numbness in the thumb, index and middle fingers, with the ring and little fingers spared, together with reduced light-touch sensation and difficulty bringing her thumb across, may indicate acute compression of the median nerve at the wrist. That’s a recognised complication after a distal radius fracture and its surgery, and it is time-sensitive, because prolonged compression can cause lasting sensory and motor loss.

“Several findings could be contributing: marked finger swelling, a tight circumferential bandage and her arm hanging down overnight. Burning pain that is out of proportion and not relieved by oxycodone is also a red flag. I can’t tell whether this is nerve compression alone or more general pressure building in the forearm, which is why it needs medical assessment today.

“The surgeon’s ‘routine post-op’ letter was written before these symptoms started, so it doesn’t reflect her current status. Starting routine range of motion and moulding a splint over a swollen hand could delay review and add pressure. Pat’s stoicism, calling this ‘just a bit numb’, could also lead us to underestimate it.

“Occupationally, the consequences are already visible. She couldn’t do up a button, she isn’t sleeping, and reduced sensation in her dominant thumb and fingers affects her ability to safely hold hot drinks, handle small objects and lift or care for a toddler. With altered sensation, a recent fracture and opioid medication, I don’t think she should be driving, though that’s the treating team’s call.

“So my immediate priority is same-day orthopaedic review; her goals of bowls, driving and minding her granddaughter are secondary today. As a student I can’t diagnose this or decide management, but I think the pattern is clear enough to escalate now.”

R Recommendation

346 words

“First, the tight bandage is now off, the backslab is still supporting her wrist, and I’d like to keep Pat here with her hand supported at about heart level rather than hanging down. I won’t fit a splint or start exercises until she’s reviewed. I’ll document the time, which fingers are affected, her response to light touch and her thumb movement, so the team has a clear baseline.

“Second, I’d like your help to contact the orthopaedic registrar now to request a same-day review for new, progressive median nerve symptoms and pain not relieved by her analgesia. If they can’t be reached quickly, we’d follow the clinic’s escalation pathway, which may mean the emergency department.

“For the MDT, I need orthopaedics to assess her nerve and circulation status and confirm fixation stability and movement and loading restrictions before any therapy starts. Nursing could repeat neurovascular observations while she waits, and the medical team or pharmacy could review her pain management. Then the hand therapist, whether an OT or a physiotherapist, can adapt the splint and programme to the surgeon’s advice.

“I’d explain to Pat and Graham why we’re waiting, and help with the parking so leaving isn’t the easiest option. I’d use teach-back on the symptoms to report urgently: increasing numbness, worsening or burning pain, colour change, or a dressing or splint feeling tight.

> "For OT assessment, once she's cleared, my principal short-term assessment would be observed performance of essential daily tasks within her precautions, such as dressing with buttons, opening her medication packaging and preparing a simple meal, noting sensation, technique, pain and swelling, because these show what she can safely do now. Longer term, I'd use the COPM to capture her own priorities, which so far include minding her granddaughter, driving and bowls, and reassess as she recovers.
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> "Intervention would include swelling management, night positioning, one-handed strategies and a temporary plan for childcare, perhaps with Graham or family doing the lifting. I'd follow up after the orthopaedic review to restart her programme. For today, Pat shouldn't leave without being seen by the orthopaedic team."

Coaching notes *not spoken*

The immediate priority

New, progressive numbness in a median nerve distribution with burning pain unrelieved by opioids, eight days after plate fixation, needs same-day orthopaedic review. Routine splinting and range of motion should wait.

Distractors in this case

- **"Routine post-op, commence ROM"** — the letter predates the symptoms; it cannot clear a new neurological change.
- **Pat's stoicism ("just a bit numb")** — minimising language from a stoic client is a reason to examine more carefully, not less.
- **Parking, the waiting patient and Pat's wish to leave** — time pressure is real but does not change the clinical priority; solve the practical problem instead.
- **Driving, bowls and minding her granddaughter** — genuine goals for the COPM later, not today's decision.

Missing information to clarify

- Pre-operative sensation and any earlier symptoms in the hand.
- Neurovascular findings since discharge and what Pat was taught to report.
- The surgeon's specific movement, splinting and loading restrictions.
- Her current analgesia use and who applied the crepe bandage.

Useful references

- American Academy of Orthopaedic Surgeons, *Management of Distal Radius Fractures: Evidence-Based Clinical Practice Guideline* (2020).
- Niver GE, Ilyas AM. Carpal tunnel syndrome after distal radius fracture. *Orthopedic Clinics of North America* (2012).

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